Typhoid (Enteric Fever) Medical Report

Sample document filled with dummy data for project analysis. Every name, number and date below is fictional and must not be used as a real medical record.

1. Patient Details

Field	Details
Name	Rahul Sharma (dummy)
Age / Sex	28 years / Male
Patient ID / UHID	UHID-2026-004517
Address	12, Sample Nagar, Anytown, 000000
Contact number	98XXXXXX01
Date of consultation / admission	12/10/2026
Referring doctor (if any)	Dr. S. Mehta (General Practitioner)
Hospital / Clinic	Sample Multispeciality Hospital, Anytown

2. Presenting Complaints

- Fever for 7 days: step-ladder pattern, maximum recorded temperature 103.4 °F
- Associated symptoms: headache, malaise, loss of appetite, abdominal pain, constipation followed by loose stools, dry cough, body ache; no rash noticed
- Relevant history: ate street food 10 days before onset; no recent travel; no known typhoid contact; no previous typhoid or typhoid vaccination

3. Past Medical History

No diabetes, hypertension or gallbladder disease. No known drug allergies. Not on regular medication. Took paracetamol 500 mg for 5 days at home; no antibiotics before presentation.

4. Clinical Examination

Parameter	Finding
General condition	Toxic-looking, mildly dehydrated
Temperature	102.8 °F
Pulse	78 /min (relative bradycardia for the fever)
Blood pressure	108/70 mmHg
Respiratory rate / SpO2	18 /min, 98% on room air
Tongue	Coated, dry
Abdomen	Mild diffuse tenderness, mild distension, splenomegaly (1 cm below costal margin), no guarding
Skin	Rose spots absent
Other systems	Chest clear, CVS normal, CNS no focal deficit

5. Investigations

Test	Result	Reference / Remarks
Hemoglobin	11.8 g/dL	13.0–17.0 g/dL; mildly low
Total leukocyte count	3,800 /µL	4,000–11,000 /µL; leukopenia is typical
Differential count	N 58%, L 36%, M 5%, E 1%	Relative lymphocytosis
Platelet count	145,000 /µL	150,000–410,000 /µL; borderline low
Blood culture (Salmonella Typhi / Paratyphi)	Positive: Salmonella Typhi isolated	Gold standard; collected before antibiotics
Widal test (O and H titres)	TO 1:160, TH 1:320	Supportive only

Test	Result	Reference / Remarks
Typhoid IgM rapid test (e.g., Typhidot)	Positive	Supportive only
Liver function tests	AST 68 U/L, ALT 74 U/L, total bilirubin 0.9 mg/dL	Mild transaminase rise
Serum creatinine / electrolytes	0.9 mg/dL; Na 136, K 3.9 mmol/L	Normal
CRP	42 mg/L	Raised
Malaria / dengue screen	Negative	Other causes of fever excluded
Ultrasound abdomen (if done)	Mild hepatosplenomegaly; no free fluid	No perforation sign
Antibiotic sensitivity	Sensitive: ceftriaxone, azithromycin. Resistant: ciprofloxacin	Fluoroquinolone resistance noted

6. Diagnosis

Enteric fever (Typhoid fever), culture-confirmed

Severity: Uncomplicated

Complications, if any: None; mild reactive hepatitis only

7. Treatment Given

Item	Details
Setting	Inpatient, general ward, 12/10/2026 to 19/10/2026
Antibiotic	Ceftriaxone 2 g IV once daily for 10 days, chosen on culture sensitivity
Antipyretic	Paracetamol 650 mg orally every 6 hours when temperature is above 100 °F

Item	Details
Fluids / supportive care	IV normal saline for 48 hours, then oral rehydration and soft diet
Other medications	Pantoprazole 40 mg once daily; ondansetron 4 mg as needed
Response to treatment	Fever settled on day 5 of treatment; appetite and abdominal pain improved; discharged afebrile

8. Advice and Recommendations

- Complete the full prescribed course of antibiotics, even after fever settles.
- Take a soft, easily digestible, adequate-calorie diet with plenty of safe fluids; avoid raw or street food.
- Drink boiled or purified water; wash hands thoroughly with soap before eating and after using the toilet.
- Rest for 7 days; fit to resume work from 26/10/2026, subject to review.
- Return immediately for persistent high fever, severe abdominal pain, black or bloody stools, vomiting, confusion, or breathlessness.
- Food handlers: avoid handling food until stool cultures are negative, as advised by the physician.
- Follow-up on 02/11/2026, with repeat CBC and liver function tests.
- Typhoid vaccination is advised after recovery for prevention of future infection.

9. Opinion / Remarks

Illness lasted 14 days in total. Patient is advised 7 days of rest and a light diet; fit to resume duties from 26/10/2026.

10. Declaration

I certify that the above information is based on my clinical examination and the investigations listed.

Doctor's name: Dr. A. Verma (dummy)

Qualification and registration no.: MD (Medicine); Reg. No. XX-00000

Signature and hospital stamp:

Date and place: 19/10/2026, Anytown